



Valuing the Hub Function

Crediting support for other institutions' research

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Where we stand

Not one hospital's site visit. We speak for a national system and ask how it should measure a research hub.

01 **UTokyo**

Director of the ARO at UTokyo Hospital, itself a Core Clinical Research Hospital.

02 **NUH-CRPI**

Representative Secretary of the council of all national university hospitals: 42 universities, 44 hospitals.

03 **J3C, AMED**

Vice Chair, Japan CDISC Coordinating Committee. AMED PI on data standards.

Japan's networks, and why a CTSA hub

Japan has national hospital networks but no hub program. We came to see how one hub serves a whole state.

01 **42 + 44**

The 42 university hospitals form a council. It shares practice; it is not a hub.

02 **NHO 140**

NHO runs 140 hospitals with its own ARO and data center.

03 **CCTSI**

One hub serving universities, hospitals and communities statewide.

Japan's Core Clinical Research Hospitals

A legal status, not a grant. The Minister approves it; 16 hospitals hold it today.

- 01 **Since 2015** Created in 2015. Approval and renewal follow published criteria.
- 02 **A status** Unlike a CTSA award, a status the hospital keeps until it fails the criteria.
- 03 **No office** No national program office sets goals. The criteria are the whole instrument.

What the criteria count

Three-year counts, in principle in-house, decide approval. Support to others is a floor, not a score.

01 8 trials

Drug-approval trials, or 4 plus 40 others. In principle led by the hospital's own PI.

02 45 papers

Peer-reviewed English papers. In principle the first author is from the hospital.

03 15 supports

A fixed floor. Beyond it, only full support counts: protocol, DM and monitoring.

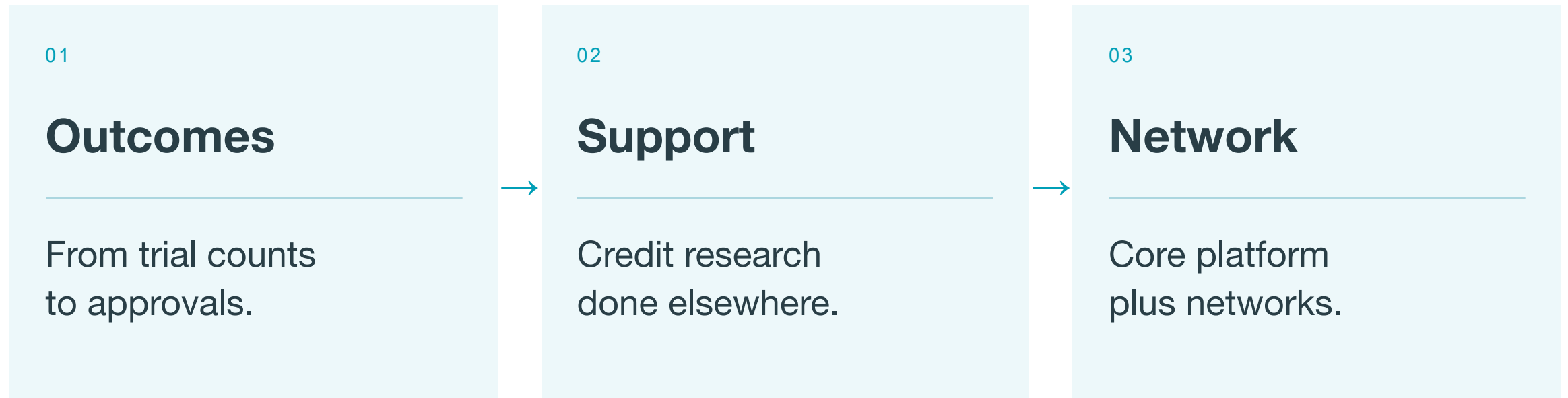
What happens as a result

Counting in-house output creates an incentive that works against the hub function.

- 01 **Small trials** Hospitals launch small, easy-to-start trials and multiply papers to meet the thresholds.
- 02 **Penalty** Every person or dollar moved to supporting others lowers the hospital's own metrics.
- 03 **Floor stays** The 2026 review keeps support as a floor; its effort was judged too hard to score.

Three shifts we propose

Re-weight, do not abolish. Each shift has a working US counterpart in how NIH funds hubs and functions.



Measure what a hub enables, not what it keeps in-house.

Measured by our own yardstick

Apply the proposed axis to ourselves. None of this counts today; all of it enabled others' research.

- 01 **Ptosh** EDC platform for academic trials; CDISC encoded at data entry.
- 02 **NHO** CDISC adopted across the National Hospital Organization's multicenter trials.
- 03 **aCRF.jp** Open annotated CRF library, and the JASPEHR group on EHR-to-research data.

What we came to ask

Thirteen written questions in seven themes; you have grouped the answers for 10:10. DCRI sharpened three.

01 **Value**

Which numbers NCATS reacts to; how partners' research is credited (A–C).

02 **People**

Support workforce and retention; at DCRI, culture and turnover under 6% (D).

03 **System**

How the hub relates to the AROs around it, and to data standards (E–G).

What we most want to learn

One question is about the review; two are specific to you.

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|----|-------------------|--|
| 01 | Review | What decided your last renewal; which report numbers has NCATS acted on? |
| 02 | If counted | If in-house counts decided hub status, how would CCTSI and its partners change? (Q3) |
| 03 | Hub, ARO | At Duke the CTSA hub and DCRI barely meet. How do CCTSI and CPC relate? |



Thank you

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